

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

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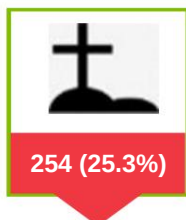
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°37/MVB_20/06/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (34)	• Ituri (22/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa
Reporting date	June 20, 2026
Publication date	June 21, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



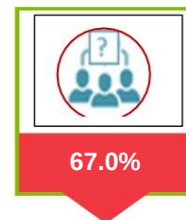
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- **Forty-seven (47) new confirmed cases, including 7 deaths**, were reported at the end of the day on June 20, 2026, in the provinces of **Ituri (42, including 4 deaths)** and **North Kivu (5, including 3 deaths)**. These confirmed cases are distributed in the health zones of **Bunia (16), Mongbwalu (11), Rwampara (10) and Bambu (5)** in the province of Ituri and in the health zones of **Katwa (2), Butembo (2) and Beni (1)** in the province of North Kivu.
- **Eight (8) patients** with Ebola virus disease have been declared cured after receiving tests of negative tests, including **6 in Ituri** and **2 in South Kivu**.
- A technical meeting was held, chaired by His Excellency the Minister of Health, Hygiene and Social Welfare, with the Chief Medical Officers of the health zones of the province of Ituri, as well as technical and financial partners.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak.

was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

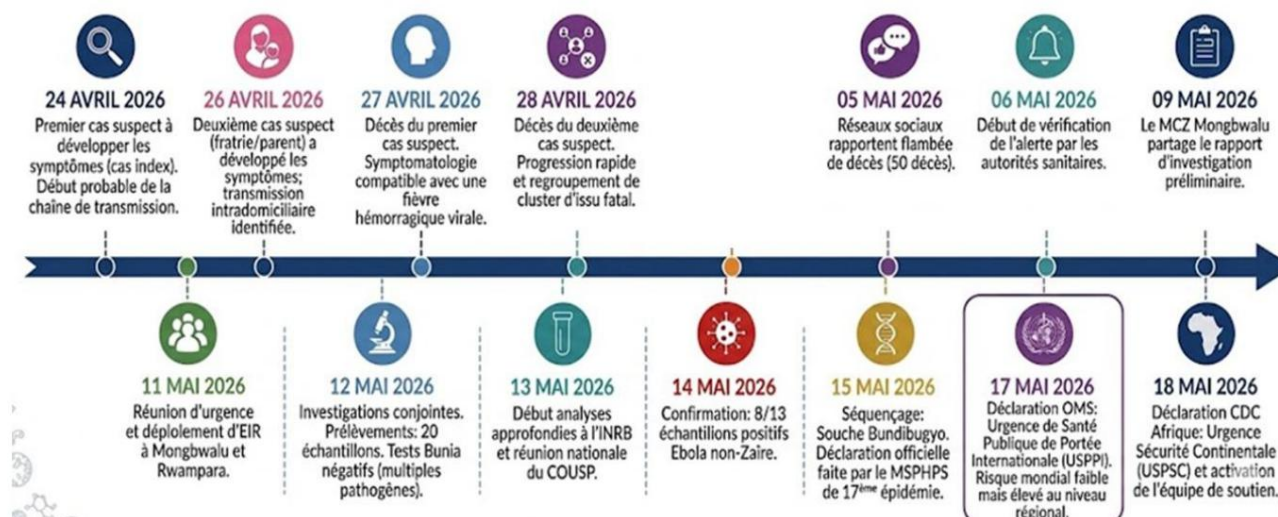


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 20, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 20, 2026.

Province	Confirmed cases	Death (confirmed)	Lethality	Affected health zones	New case confirmed (24h)
Ituri	916	205	22.4%	22 out of 36 (61.1%)	42
North Kivu	84	48	57.1%	11 out of 34 (32.3%)	5
South Kivu			33.3%	1 out of 34 (2.9%)	0
Total	3 1003	1 254	25.3%	34 out of 104 (32.7%)	47

No new health zones were affected by the end of June 20, 2026. Thus, the number of affected health zones remains at 34/104 (section 3.1). The province of South Kivu reported its last confirmed case on May 26, 2026, suggesting an absence of recent transmission. However, active surveillance remains necessary.

3.2 Distribution by health zone (Cumulative as of 20/06/2026)

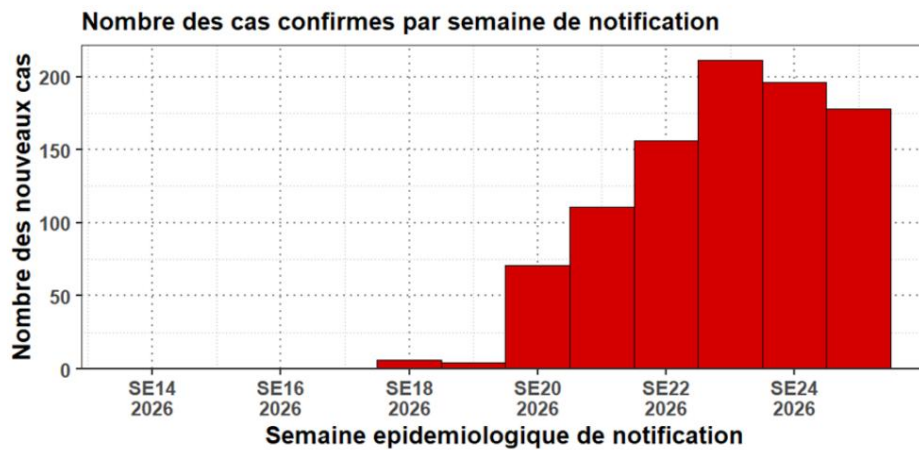
- **Ituri Province (916 cases):** Bunia (275), Rwampara (211), Mongbwalu (220), Nyankunde (79), Nizi (26), Lita (20), Bambu (14), Komanda (10), Kilo (7), Mangala (7), Tchomia (5), Damas (4), Aungba (6), Rimba (3), Aru (3), Mambasa (2), Logo (2), Drodoro (1), Gety (1), Kambala (1), Nia-Nia (1) and Fataki (1).

Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.

- **North Kivu Province (84 cases):** Katwa (28), Butembo (27), Beni (17), Oicha (3), Kalunguta (2), Kyondo (2), Goma (1), Masereka (1), Vuhovi (1), Mabalako (1) and Musienene (1).

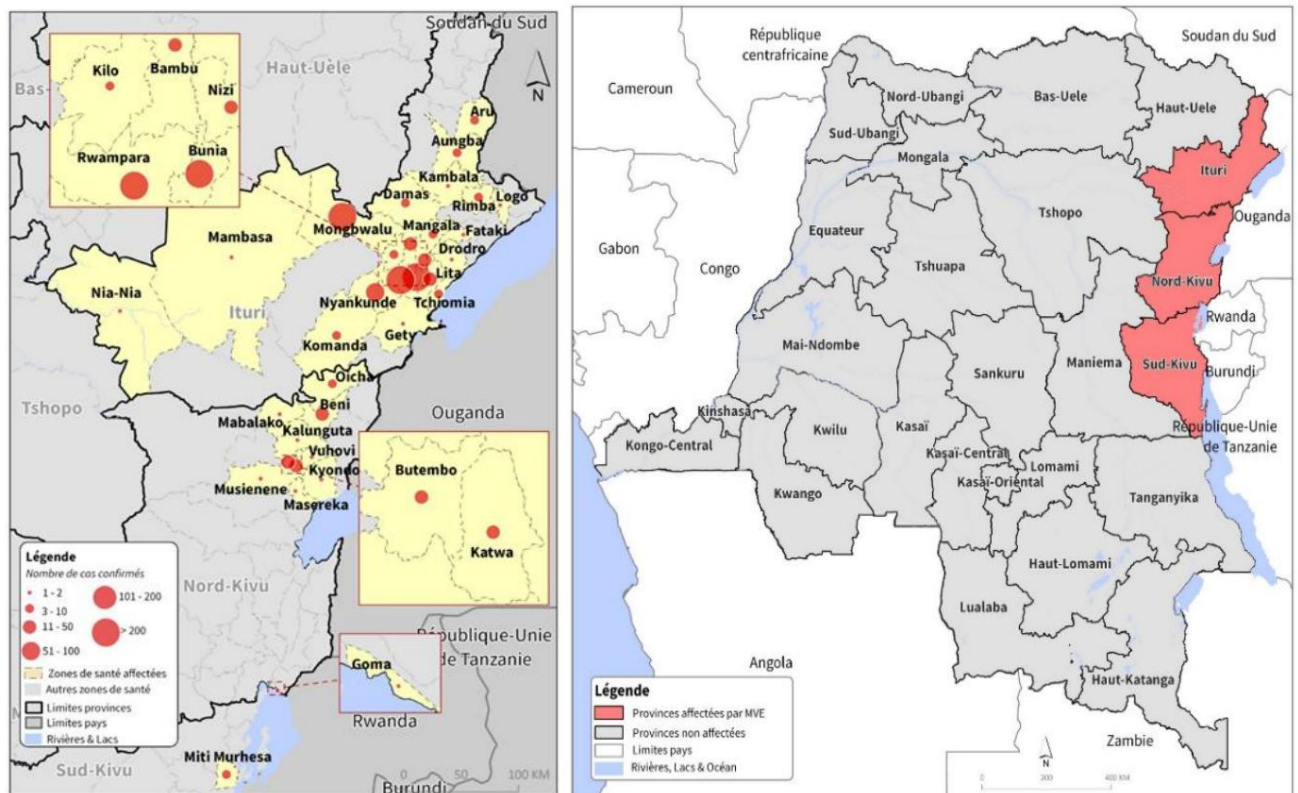
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Temporal Analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting continuous transmission of the disease within the community. Rapid geographical expansion is also observed. The epidemic remains possible if public health actions are not implemented quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 933.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 20, 2026

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (June 20, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Lethality (CFR, %)
ITURI	916	205	22.4%
Bunia	275	44	16.0%
Rwampara	211	35	16.6%
Mongbwalu	220	91	41.4%
Nyankunde	79	7	8.9%
Nizi	26	5	19.2%
Lita	20	6	30.0%
Komanda	10	3	30.0%
Bamboo	14	2	14.3%
Kilo	7		14.3%
Mangala	7	1	57.1%
Tchomia	5	4	0.0%
Damascus	4	0	0.0%
Aungba	6	0	33.3%
Rimba	3	2	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	2	0	0.0%
Drodro			100.0%
Gety		1	0.0%
Kambala			100.0%
Nia-Nia	1111	101011	100.0%
Fataki	1	0	0.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	84	48	57.1%
Butembo	27	13	48.1%
Katwa	28	17	60.7%
Blessed	17	12	70.6%
Oicha	3	2	66.7%
Kalunguta	2	1	50.0%
Kyondo	2	1	50.0%
Goma		0	0.0%
Masereka	11	0	0.0%
Vuhovi	1	1	100.0%
Mabalako	1	0	0.0%
Musienene	1	1	100.0%
SOUTH KIVU	3	1	33.3%
Miti-Murhesa	3	1	33.3%
TOTAL	1003	254	25.3%

The epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo is progressing, with **47 new confirmed cases including 7 deaths** reported in the last 24 hours, bringing the total to **1,003 confirmed cases** and **254 deaths**, representing an overall case fatality rate of **25.3%**.

Provincial dynamics and case concentration

Approximately **91.3%** of cases (n=916) and **80.7%** of deaths (n=205), representing a case fatality rate of **22.4%**, are concentrated in **Ituri province**, making it the absolute epicenter of the epidemic. The most affected health zones remain Bunia (275 cases), Rwampara (211 cases), and Mongbwalu (220 cases), encompassing slightly more than three-quarters of the reported cases (n=706) in the province. The Bunia and Rwampara health zones have moderate case fatality rates (16.0% and 16.6%, respectively), while the **Mongbwalu** zone shows marked severity, with a case fatality rate of **41.4%** (91 deaths out of 220 cases), suggesting persistent challenges in early intervention and access to care. Furthermore, 17 additional cases have been reported in health zones not yet identified, with no associated deaths, which could indicate a geographical spread.

requiring further investigation.

Severity of the epidemic in North Kivu

A total of **84 confirmed cases** and **48 deaths**, with a provincial case fatality rate of **57.1%**, has been reported in **North Kivu** province. This province presents the most concerning profile in terms of severity, exceeding the national average. This high case fatality rate is driven by the health zones of **Beni** (70.6%), **Katwa** (60.7%), and **Oicha** (66.7%). The Katwa health zone has the highest reported case fatality rate.

number of cases (n=28), followed by the Butembo ZS (27) which has a slightly lower lethality (48.1%).

The presence of cases with a 100% lethality rate in areas with low population numbers (Vuhovi, Musienene) is an indicator of the danger of the virus in this region and the need for an immediate clinical response.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Visit of His Excellency the Minister of Health, Hygiene and Social Welfare, accompanied by the Governor of province in the Mongbwalu health zone.
- Holding a technical meeting with the Chief Medical Officers of the health zones in Ituri province as well as technical and financial partners.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 20, 2026

Indicator		Ituri North Kivu		South Kivu	Total
Alerts raised		393	436	11	840
Alerts investigated		393	436	10	839
Investigation rate		100.0%	100.0%	90.9%	99.9%
Alerts confirmed	Alive	125	41	5	171
	Deceased	21	9	0	30
Total suspected cases for today		146	50	5	201

For June 20, 2026, **840 alerts** were reported by the three provinces, **839 (99.9%)** of which were 201 suspected cases investigated, **including 30 deaths**, were identified (Table 3).

Table 4. Contact tracing of confirmed cases as of June 20, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	5,920	4056	68.5%
North Kivu	1935	1187	61.3%
South Kivu	58	58	100.0%
Total	7,913	5,301	67.0%

- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- 171 new contacts** have been listed in 9 health zones in Ituri province. These contacts are distributed in the ZS of Rwampara (54), Mangala (48), Bunia (23), Nyankunde (15), Aungba (15) Mongbwali (3), Aru (5), Mambasa (4), Nizi (4).

Update on checkpoint and point of entry (PoC/PoE) activities

- Activation of PoC Boga in the Rubingo Health Area, Boga Health Zone.
- Four (4) alerts were notified, including 3 lifeless bodies intercepted at the Mudzibala PoC (2 of which were accompanied by the EDS team from the Rwampara ZS without swab documents).
destination of the cemetery) and 1 live alert **at the Foner Mambasa PoC** which has been validated.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) in Ituri province, as of June 20, 2026

INDICATORS	Ituri	North Kivu	Kinshasa	Overall
Report completeness	100% (44/44)	100% (12/12)	100% (4/4)	100% (60/60)
Proportion of PoC enabled and PoE enhanced	80% (44/55)	92.3% (12/13)	100% (4/4)	83.3% (60/72)
Number of people who have switched to PoE/PoC	83,708	43,021	6,825	133,554
% of travelers screened	98.0%	97.1%	92.9%	96.0%
% of travelers who washed their hands %	97.5%	97.1%	90.8%	97.2%
of travelers made aware	95.3%	97.1%	90.7%	94.4%
Number of alerts notified	4	0	0	4
Number of validated live alerts	1	0	0	1
Number of lifeless bodies intercepted today	3	0	0	3
Number of problematic contacts intercepted	0	0	0	0
today % of validated live alerts referred to CT/CTE	0	0	0	0
% of lifeless bodies swabé	0	0	0	0
Number of positive results of referred suspected cases	0	0	0	0

4.3. Laboratory

- **Ituri:** 119 samples collected and analyzed (positivity 35.2%; n=42).
- **North Kivu :** 39 new samples collected and analyzed (positivity 12.8%; n=5).
- **South Kivu :** 5 new samples were collected and all came back negative for Ebola virus disease after analysis.

4.4. Infection Prevention and Control (IPC)

Ituri: A briefing of 39 service providers was organised in the Bambu and Bunia health zones on injection safety, waste management and standard IPC precautions (correct hand washing, wearing and removing PPE, as well as bio-cleaning); decontamination carried out around confirmed cases in the Nia-Nia, Mambasa and Rwampara health zones; 20/21 EDS carried out in the Rwampara (7/8), Bunia (6/6) and Mongbwalu (7/7) health zones.

North Kivu : PCI Scorecard evaluation of 8 ESS (average PCI score: 53.9%) followed by a briefing of 62 healthcare providers; completion of 5 EDS in 24 hours; 8 swabs carried out on death alerts reported by the morgue.

4.5 Holistic care

- **Eight (8) patients** with Ebola virus disease have been declared cured after receiving negative control tests including 6 in Ituri and 2 in South Kivu.
- Continued nutritional monitoring of patients in the CTEs: 630 hot meals were served, including 87 among confirmed cases, 143 among suspected cases, 344 among PPL and 56 among accompanying persons.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 20, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	307	54	12	373
Total admissions (24h)	62	4	1	67
Exits (24h)				
Deceased (Suspects/Confessions)	21	0	0	21
No case	34	6	0	40
Cured	6	0	2	8
Escapees (Suspects/Confessions)	6	0	0	6
Transferred to the HGR	0	0	0	0
Total outflows	67	6	2	75
Patients in isolation (End of Day),	302	52	11	365
including confirmed cases	139	24	0	163
(NC+AC) and suspected cases	163	28	11	202
Overall occupancy rate	89.3%	73.2%	34.4%	82.8%

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 20, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	13 (46.4%)	0	0
Confirmed cases currently being monitored that have received interventions SMSPS	50 (37.9)	5 (100%)	0
New suspected cases that benefited from SMSPS interventions	9 (13.4)	0	0
Suspected cases under follow-up that have received interventions SMSPS	31 (18.7)	18 (100%)	4 (100%)
Laboratory results have been announced.	28	0	5
Including positive ones	13	0	0
Number of children separated in daycare who benefited from interventions SMSPS	8 (100%)	0	1 (100%)
Number of children separated in the community who benefited from SMSPS interventions	1	0	16
Number of accompanying persons at the CTE who benefited from interventions SMSPS	38	11	2
Number of PPLs who benefited from SMSPS interventions	3	5	23
Number of household and community members who benefited from SMSPS interventions	19	25	0
Number of EDS supported	3	0	0

4.6 Continuity of care

- Training in progress for 40 midwives and nurses from the Nyankunde and Rwampara health zones in sexual and reproductive health in the context of Ebola.
- Monitoring of drug supplies in 3 HGRs.
- Effective implementation of free healthcare with the support of the central government and its partners.

4.7 Risk communication and community engagement

Ituri: Mass awareness campaign targeting 685 people on compliance with Ebola prevention measures, care, reporting of community alerts and EDS in the health zones of Bunia, Aungba, Nyankunde.

Bambu, Nizi, Nia-Nia and Rwampara; 2,235 households reached during home visits supported by 350 Recos and 5,767 people (2,345 men and 3,422 women) reached with messages on the measures

Ebola prevention and alert reporting; educational talks by 17 Regional Health Officers (RHOs) with 5,842 people on preventive measures and the benefits of going to the Ebola Treatment Center (ETC) early; briefing of 160 service providers on community-based surveillance, organization of home visits, facilitation of educational talks in their respective communities and use of tools (community definition of cases, contact registration form, contact tracing form and home visit form) in the health zones of Bunia and Nia-Nia; continued dissemination of spots and messages on Ebola prevention measures in 18 media outlets; a radio program on infodemics and community feedback.

Support for other pillars : Surveillance: Listing around a recovered case who still presents signs of Ebola virus disease (EVD), and the patient is transferred to the Rwampara Ebola Treatment Center (ETC); support for the investigation of 5 new admissions to the ETC.

CME; facilitation of an investigation and contact listing at the TOKOBIKA hospital center; Support for the investigation in NGANA in 3 structures; facilitation of pre-listing around a suspected case in TCHUNGA 2.

PCI : Preparation of a family for household decontamination. **Result** : Decontamination completed; support for the EDS at the CTE-CME. Result: 3 EDSs completed. **North Kivu**:
Facilitation of 8 swabs and 5 EDSs.

4.8. Logistics

- Delivery of IPC supplies and medicines to the health zones of Kilo, Nyankunde, Kilo General Referral Hospital, Nizi General Referral Hospital, and other health zones Bambu, HGR Mongbwalu, CTE Mongbwalu, CTE Elikya, CTE CME Bunia, HGR Rwampara, CTE Rwampara.
- Delivery of PCI inputs to the safety and security pillar.
- Delivery of supplies to the offices of the Red Cross, WHO and DPS Ituri.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Continued awareness-raising efforts among communities and stakeholders involved in the response.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity in CTE and saturation in Ituri (82.8%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3 Poor performance in contact tracing (67.0% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4. Low reporting of alerts in certain affected zones	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not localized or unclassified (17 cases)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (78 cases) HCW (including 18 deaths, case fatality rate 23.1%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (57.1%)	Deploy a laboratory catch-up plan, strengthen diagnostic capacities

9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13.	Continuity of Essential Services (CEHS) Compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

Official ceremony for the discharge of two patients cured of Ebola virus disease, at the Lwiro Ebola Treatment Center, Miti-Murhesa Health Zone, South Kivu province.



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